# Cardiopulmonary Resuscitation (CPR) Policy

| | |
|---|---|
| Post holder responsible for Procedural Document | Rebecca Appelboam, Clinician Lead for Resuscitation |
| Author of Policy | Donna Seccombe, Senior Resuscitation Officer |
| Division/ Department responsible for Procedural Document | Safety and Risk |
| Contact details | x8967 |
| Date of original policy | 01/10/2005 |
| Impact Assessment performed | **Yes**/No |
| Ratifying body and date ratified | Safety and Risk Committee: 4 April 2019<br>Mycare amends February 2020 |
| Review date (and frequency of further reviews) | December 2021 (every 2 ½ years) |
| Expiry date | June 2022 |
| Date document becomes live | 21 June 2019 (v.6.0 9 October 2020) |

Please *specify* standard/criterion numbers and tick ✓ other boxes as appropriate

| **Monitoring Information** | | **Strategic Directions – Key Milestones** | |
|---|---|---|---|
| Patient Experience | ✓ | Maintain Operational Service Delievery | |
| Assurance Framework | | Integrated Community Pathways | |
| Monitor/Finance/Performance | | Develop Acute Pathways | |
| CQC Regulations/Outcomes: | **Outcome 4,11,12,13** | Infection Control | ✓ |
| Other *(please specify)*: | | | |
| **Note:** This policy has been assessed for any equality, diversity or human rights implications | | | |

**Controlled document**
This document has been created following the Royal Devon and Exeter NHS Foundation Trust Policy for Procedural Documents. It should not be altered in any way without the express permission of the author or their representative.

| Full History | | Status: Final | |
|---|---|---|---|
| Version | Date | Author (Title not name) | Reason |
| 1.0 | 10/10/2007 | Resuscitation Training Officer | Replacing existing Resuscitation policy 2005 |
| 2.0 | 01/10/2011 | Senior resuscitation Officer | Minor updates |
| 3.0 | 01/10/2012 | Senior resuscitation Officer | Reviewed & updated |
| 4.0 | December 2014 | Senior resuscitation Officer | Complete review of Resuscitation Policy and incorporation of Early Warning System Procedure 2011, and Resuscitation Training Policy |
| 5.0 | January 2019 | Clinical Lead for Resuscitation | Complete review |
| 6.0 | February 2020 | Clinical Lead for Resuscitation | MyCare ready |

| | |
|---|---|
| **Associated Trust Policies/Procedural documents:** | Emergencies in Non Clinical Areas (inc. carparks) Policy<br>Infection Prevention and Control Policy<br>Advance Decision to Refuse Treatment Policy<br>End Of Life Care Policy<br>Mental Capacity Act Practice Guidance |
| **Key Words:** | Resuscitation, Resus trolley / grab bag, TEP, BLS, life support courses eg ALS, Defibrilation, NEWS2, Deteriorating Patient, Resuscitation Equipment, AED, Cardiac arrest, anaphylaxisis, maternal cardiac arrest, paediatric cardiac arrest, neonatal cardiac arrest cardiac arrest in the community. |
| **In consultation with and date:**<br>Governance Managers – 11/03/2019<br>Associate Medical Directors 11/03/2019<br>Assistant Directors of Nursing– 11/03/2019<br>Senior Nurses (circulated by email) – 11/03/2019<br>Divisional Directors (circulated by email) – 11/03/2019<br>Cluster Managers (circulated by email) –11/03/2019<br>General Managers (circulated by email) – 11/03/2019<br>Resuscitation Group (circulated by email) – 11/03/2019<br>Patient Safety Group (circulated by email) – 11/03/2019<br>Quality Assurance –20/06/19<br>Safety & Risk Committee – 04/04/2019<br>Safeguarding 25/04/2019<br>Resuscitation Group on 26th March 2019<br>Safety and Risk Committee on 4th April 2019 | |
| **Contact for Review:** | Senior Resuscitation Officer |
| **Executive Lead Signature:**<br>*(Only applicable for Strategies & Policies)* | Adrian Harris, Medical Director |

## CONTENTS


KEY POINTS OF THIS POLICY ..... 4
1. INTRODUCTION ..... 5
2. PURPOSE ..... 5
3. DEFINITIONS ..... 5
4. DUTIES AND RESPONSIBILITIES OF STAFF ..... 6
5. EDUCATION AND TRAINING ..... 7
6. RECOGNITION AND ESCALATION OF THE DETERIORATING PATIENT ..... 8
7. RECOGNITION AND ESCALATION OF DETERIORATING PATIENTS – COMMUNITY SITES ..... 10
8. RESUSCITATION EQUIPMENT – PROCUREMENT / REPLENISHMENT TROLLEYS / GRAB BAGS ..... 10
9. CARDIAC ARREST WONFORD SITE ..... 13
10. CARDIAC ARREST AND MEDICAL EMERGENCY TEAM LEADER ..... 14
11. ANAESTHETIC COVER FOR CARDIAC ARRESTS ..... 14
12. PAEDIATRIC ARREST ..... 14
13. NEONATAL EMERGENCY ..... 15
14. MATERNAL CARDIAC ARREST TEAM ..... 15
15. CARDIAC ARREST COMMUNITY SITES ..... 15
16. MEDICAL EMERGENCIES AND CARDIAC ARREST IN NON-CLINICAL AREAS 16
17. SPECIAL CIRCUMSTANCES ..... 17
18. TREATMENT ESCALATION PLAN & RESUSCITATION DECISIONS ..... 17
19. RELATIVES WITNESSING RESUSCITATION ATTEMPTS ..... 19
20. POST RESUSCITATION CARE AND PATIENT TRANSFERS ..... 19
21. SAFETY ..... 20
22. ARCHIVING ARRANGEMENTS ..... 21
23. PROCESS FOR MONITORING COMPLIANCE WITH AND EFFECTIVENESS OF THE POLICY ..... 21
24. REFERENCES ..... 22
25. FURTHER INFORMATION ..... 23
APPENDIX 1: COMMUNICATION PLAN ..... 24
APPENDIX 2: EQUALITY IMPACT ASSESSMENT TOOL ..... 25

## KEY POINTS OF THIS POLICY

Healthcare organisations have an obligation to provide a high-quality resuscitation service and ensure that staff are trained and updated regularly and with appropriate frequency to a level of proficiency appropriate to each individuals expected role (Resuscitation Council UK)

This policy provides staff with guidance on the responsibilities of the organisation, resuscitation service and staff in the recognition, escalation and management of deteriorating patients and in the management of a cardiopulmonary arrest. This guidance is as recommended by the Resuscitation Council (UK). This policy applys to all staff within the organisation in the acute and community setting.

The Key points within this policy are regarding

- Recognition and escalation of deteriorating patients in the acute and community setting using the NEWS2 track and trigger process.

- What to do in the event of a cardiopulmonary arrest.

  All patients, visitors and staff within the Trust will receive resuscitation in the event of a cardiopulmonary arrest unless they have been designated 'not for an attempt at cardiopulmonary resuscitation' by their medical team (Treatment Escalation Plan / TEP in place) or have an appropriate advanced directive.

  The treatment carried out during resuscitation is that recommended by the Resuscitation Council (UK) and the European Resuscitation Council, whose guidelines are followed for adult, paediatric and neonatal resuscitation.

  As a minimum staff will be able to recognise cardiac arrest and summon help. They will instigate Basic Life Support as per current Resuscitation Council (UK) guidance and if appropriate deliver a shock using an Automated External Defibrillator (AED).

- Education and training. This policy applies to all staff working within the Trust. They are responsible for ensuring they attend the required level of training and required updates identified for their staff group, in conjunction with their line manager.

- Resuscitation equipment. There should be standardisation of equipment across Trusts and departments to facilitate use in medical emergencies. Resuscitation trolleys and grab bags will be standardised accepting that some areas will require adjustments. Adjustments will only be made by the Resuscitation Service.

## 1. INTRODUCTION

1.1 Cardiopulmonary resuscitation should be performed on any individual in the event of cardiopulmonary arrest throughout the Royal Devon and Exeter Foundation Trust. This relates to the acute and community setting; unless there is Treatment Escalation Plan or an Advanced Decision to Refuse Treatment stating that the individual is not for resuscitation. All clinical staff and some administrative staff with patient contact will be trained to a minimum of Basic Life Support as recommended by current Resuscitation Council (UK) Guidance.

1.2 **Failure to comply with this policy could result in disciplinary action.**

## 2. PURPOSE

2.1 The purpose of this policy is to provide guidance for the planning and implementation of a high-quality resuscitation service to the Trust (acute and community), whilst supporting all Trust activity with regard to issues around resuscitation, recognition of and response to deteriorating patients, and training requirements for staff.

2.2 The policy incorporates the current published guidelines for resuscitation by the Resuscitation Council (UK).

## 3. DEFINITIONS

3.1 **CPR** - Cardiopulmonary Resuscitation is an emergency procedure in which the heart and lungs are made to work by manually compressing the chest overlying the heart and forcing air into the lungs. CPR is used to maintain circulation when the heart stops pumping, usually because of disease, drugs, or trauma.

3.2 **BLS** – Basic Life Support refers to maintaining airway patency and supporting breathing and the circulation without the use of equipment other than protective devices.

3.3 **ALS** – Advanced Life Support refers to emergency medical procedures in which basic life support efforts of cardiopulmonary resuscitation are augmented by establishment of an intravenous (IV) fluid line, possible defibrillation, drug administration, control of cardiac arrhythmias, endotracheal intubation, and use of ventilation equipment. Term also refers to a resuscitation council accredited course.

3.4 **AED** - Automated External Defibrillator is a portable device that analyses the heart rhythm. If required it can deliver an electric shock to the heart to try to restore a normal rhythm.

3.5 **IMCA** - Independent Mental Capacity Advocate is a role created by the Mental Capacity Act 2005. A local council or NHS body has a duty to involve an IMCA when an unbefriended person aged 16 + who lacks mental capacity to understand a decision which needs to be made about serious medical treatment. They will represent the patient and act in the person's best interests.

3.6 **ROSC** - Return of Spontaneous Circulation following a successful resuscitation attempt.

3.7 **TEP** – Treatment Escalation Plan details the resuscitation status and the ceiling of care for patients.

3.8 **NEWS2** – National Early Warning Score is calculated every time a patient's physiological observations are undertaken. https://www.rcplondon.ac.uk/projects/outputs/national-early-warning-score-news-2

3.9 **DNACPR** – Do Not Attempt Cardiopulmonary Resuscitation in the event of the person suffering a cardiac arrest.

3.10 **ADRT** – Advanced Decision to Refuse Treatment. Formerly known as Advanced directives or Living Will.

3.11 **LPA** – Lasting Power of Attorney. A person nominated to make decisions on behalf of a person if they are unable to make decisions due to lack of capacity.

3.12 **ILS** – Immediate Life Support is a Resuscitation Council UK accredited course.

3.13 **BPA** – Best practice advice

## 4. DUTIES AND RESPONSIBILITIES OF STAFF

4.1 **The Medical Director** has overall responsibility for the effective implementation of this policy throughout the Trust.

4.2 **The Safety and Risk Committee** are responsible for receiving assurance from the Patient Safety Group on all CPR matters. The committee will ratify the policy and ensure policy distribution, implementation and compliance throughout the organisation / Trust.

4.3 **Patient Safety and Mortality Group** receive quarterly assurance reports form the Resuscitation group. Where it will review and note the report. The Patient Safety and Mortality group report to the Safety and Risk committee.

4.4 **Resuscitation Group** has overall responsibility for the implementation, evaluation and monitoring of the cardiopulmonary resuscitation policy throughout the Trust and will report to the Patient Safety Group

The Resuscitation Group must include representatives from stakeholder groups, as recommended in the quality standards for cardiopulmonary resuscitation practice and training in the acute care setting, (Resuscitation Council, 2018)

The Chair of the Resuscitation Group must be a senior clinician with an active and credible involvement in resuscitation. This individual would be expected to have the authority to drive and implement change.

The role of the resuscitation group is to:

- Oversee policy and guideline development
- Advise the Trust on resuscitation process and equipment in line with the Resuscitation Council (UK) guidelines
- Review equipment and procedures for resuscitation, and ratify change
- Receive reports on training compliance
- Review audits on resuscitation and equipment

- Identify risks and address
- Review trends and work with Clinical Governance and Patient Safety to improve safety

The group will meet quarterly to achieve the aforementioned duties, with the appropriate administrative support required.

4.5 **Resuscitation Officers** are responsible for the resuscitation service and will ensure effective and efficient delivery of resuscitation training throughout the Trust.

4.5.1 Due to the size and geographical nature of the Trust the use of 'local' basic life support trainers (BLS Key Trainers) are advised. The Resuscitation Officers (ROs) are responsible for educating and updating these local trainers.

4.5.2 The ROs are responsible for the management of the Public access defibrillator cabinets.

4.5.3 They will attend Resuscitation group meetings and the resuscitation group will send a representative to the End of life Group meetings.

4.5.4 They will audit and maintain a database of all Cardiac Arrest calls and submit data to the NCAA .They will endeavour to collect data on MET calls but serious incidents should be reported via the Datix system as per the Trust policy.

4.5.5 The RO's will ensure standardisation of equipment across the Trust, recognising that individual areas may have the need for suitable adjustment.

4.6 **The Cardiac Arrest Team Leader** will coordinate and run the resuscitation attempts using the Resuscitation Council (UK) guidelines (link to resus council home page)

4.7 **Ward and Departmental Managers** have overall responsibility for implementing this policy within their area(s) of control.

4.8 **The Medical Equipment Management Manager (MEM)** is responsible for the maintenance and checking of equipment and should be consulted prior to new equipment being purchased for the Trust.

4.9 **All Clinical Staff** have a responsibility to update their Basic Life Support skills annually and attend advanced courses if identified as required for their role.

## 5. EDUCATION AND TRAINING

5.1 All training will be delivered following current Resuscitation Council (UK) standards and guidelines.

5.2 The Trust has formulated a matrix for guidance on required resuscitation training (including update timeframes) for all Trust staff. This will be reviewed by the Resuscitation Group and updated by the Senior Resuscitation Officer.

5.3 Staff should NOT withhold administering CPR if their current life support certificate is 'out of date'. Staff with prior instruction in BLS / ILS / ALS should administer CPR to their current scope of knowledge.

5.4 Formal resuscitation training events will be recorded, with all documented attendees being submitted for inclusion on the ESR database.

5.5 It is the responsibility of line managers / department managers to release staff for training and monitor annual updates. If problems are identified they will put in place action plans to rectify any problems.

5.6 Staff new to the Trust must receive resuscitation training during their Trust induction.

## 6. RECOGNITION AND ESCALATION OF THE DETERIORATING PATIENT

### Early warning score

6.1 The Trust utilises the National Early Warning Score (NEWS2) system of monitoring of patients' vital signs to identify deteriorating patients, based on the guidance from the Royal College of Physicians guidline. https://www.rcplondon.ac.uk/projects/outputs/national-early-warning-score-news-2

6.2 A National Early Warning Score (NEWS2) should be calculated every time patient's physiological observations are undertaken.

6.3 **Inclusions**:

- All in-patients aged over sixteen years at the Trust – Wonford, Heavitree, and in Community Hospitals
- Patient 16 years and over presenting at the Emergency Department will be scored using the NEWS2 track and trigger system.
- Any patient attending the hospital for a procedure, which necessitates their observations being taken.
- Any patient attending hospital for assessment

6.4 **Exclusions**:

- Paediatric patients have a specific paediatric scoring system.. 0-3 months, 4-11 months, 1-4 years, 5-11 years 12 years plus
- Out-patients.
- Patients where a specific plan not to escalate has been endorsed by a senior clinician and this is documented in the patient's medical records and / or Treatment Escalation Plan.
- Women who are being cared for by maternity services
- Caution should be exercised using the NEWS2 score for patients with a spinal cord injury.

6.5 All clinical staff involved in carrying out patient observations should be trained in the use of NEWS2. This will be delivered on Trust induction and on annual updates. Staff will receive training in using MYCARE to digitally record the patients vital signs correctly. There will also be e-learning available via the Hub or via this link regarding NEWS2: https://news.ocbmedia.com/

6.6 Deviations away from normal physiological parameters according to the NEWS2 will generate a score, which will trigger a recommended best practice advisory intervention for medical and nursing staff as detailed on the best practice advice (BPA) pop up which will appear on the NEWS2 chart screen

6.7 All clinical staff must be aware of their role and responsibility in the effective recognition and escalation of a patient who may be deteriorating.

6.8 The patient's NEWS2 score should be included as part of the formal handover of patients.

6.9 The frequency of when observations are carried out and recorded should follow the NEWS2 recommendation in accordance to the patient's condition. There may be circumstances when clinical judgement overrides this recommendation, and in this situation, it may be appropriate to increase frequency of observations. The minimum frequency of observations is at least 12-hourly unless receiving end of life care where it may be documented in the patients plan 'not for observations'.

6.10 Patients being transferred within the Trust should have their NEWS2 calculated prior to moving and again on arrival in the new area.

6.11 The decision to place patients on scale 2 should only be made by a senior clinician with documented evidence of hypercapnic respiratory failure (based on a blood gas from previous or same hospital admission).

6.12 Adapted parameters can be documented by a senior clinican. For example, if the patient has a condition which deviates from the considered normal range such as oliguric renal failure, or chronic respiratory disease, it may be appropriate to document an acceptable deviation. Another example might be a physiological tachycardia associated with anaemia. If the NEWS2 parameters are altered, it is important that they are reviewed frequently, and a plan made to re-adjust to the normal parameters when/if this is clinically appropriate. The default within MyCare will be to return to generic NEWS 2 parameters after 3 days.

6.13 The observation chart within MyCare has a section for the doctor to document alterations to physiological parameters.

6.14 Where a patient is recognised as being in the last few days or hours of life a personalised plan of care should be made and agreed by the patient, family and multi-disciplinary team using the dedicated documentation within relevant section of MyCare or available to download from HUB. End of Life Policy

6.15 The Resuscitation Officers will review all cardiac arrest calls (2222) and collect data for submission to the National Cardiac Arrest Audit (NCAA). They will also collect baseline process data for all Medical Emergncy Team (MET) calls, but will not carry out a detailed review of these cases – where there is clinical concern this should be escalated to the Clinical Lead for the Resuscitation Group and the Lead Nurse for Patient Safety via the Datix System, or informally to the Consultant responsible for the patients care or the Senior Nurse for the area.

6.16 All new clinical staff will receive education with regard to the National Early Warning Score NEWS2 used within the Trust in-line with the Royal College of Physicians document NEWS2. Standardising the assessment of acute-illness severity in the NHS, which has been endorsed by NHS England (December 2017).

6.17 Deteriorating patients should be escalated to the relevant teams as per the best practice advisory (BPA) from the NEWS2 escalation chart within MyCare .

6.18 The Trust uses a Medical Emergency Team (MET) to respond to acute clinical emergencies identified by clinical triggers (NEWS2 score) or other indicators.

6.18.1 The MET TEAM is made up of the following personnel:

- Medical ST3 or above (or equivalent),
- On call ICU – variable seniority, but always with access to a senior ICU opinion either via Consultant (during the day) or by 4th on call anaesthetist at night.
- Medical F2 on call
- Senior Matron on call (17:00 -21:00 Mon – Fri) and (07:30-21:00) at weekends
- Site Practitioner (during normal working hours if available, out of normal working hours and non-clinical areas only)
- Resuscitation Officers (during normal working hours, if available)

6.18.2 The MET team can be contacted via switchboard by dialling 2222 and asking for MET attendance.

6.18.3 If a patient deteriorates rapidly or collapses, Staff should summon immediate help by dialling 2222and asking for either a cadiac arrest or MET as appropriate.

6.18.4 The Medical Emergency Team (MET) is based only in the Wonford Main Building, for all other sites, if appropriate, call for an emergency ambulance, (9)999.

6.18.5 The MET team will assist the patient's **own team** to plan further care for the patient, and will provide immediate supportive care as appropriate. If a member of the patients own team does not attend the MET call, the nursing staff on the ward should ensure that they are contacted – or the on call if out of hours.

## 7. RECOGNITION AND ESCALATION OF DETERIORATING PATIENTS – COMMUNITY SITES

7.1 Patients in community hospitals should have their observations recorded and scored using the NEWS2 system as occurs in the acute Trust. For Children under the age of 16 the score should be calculated using the appropriate PEWS for their age.

7.2 If a patient is deteriorating or has a high NEWS2 or PEWS score, an emergency ambulance must be summoned using (9) 999.

7.2.1 Initiation of the appropriate emergency protocols / interventions must occur while awaiting its arrival.

7.2.2 Any interventions and relevant history must then be handed over to ambulance personnel.

7.2.3 Where resources allow, a member of the team should accompany the patient to the acute hospital.

7.3 For some patients in community hospitals, escalation of care is inappropriate, and this should be clearly documented on a treatment escalation plan (TEP) form.

## 8. RESUSCITATION EQUIPMENT – PROCUREMENT / REPLENISHMENT TROLLEYS / GRAB BAGS

8.1 The Resuscitation Group will set the parameters for equipment in each of these areas according to the minimum equipment and drug lists for cardiopulmonary

resuscitation (Resuscitation Council, 2013b). Appopriate adjustments to equipment lists may be made if required for specific clinical areas.

8.2 The Resuscitation Group will ensure that resuscitation equipment is up to date and fit for purpose by carrying out audits of equipment.

8.3 All resuscitation trolleys/ grab bags should be stocked in accordance with the latest approved checklist. The resuscitation team will provide approved checklists for all areas throughout the organisation and appropriate checking record booklets. The resuscitation team can be contacted on the generic e-mail address (rde-tr.resusofficer@nhs.net) if a new checking booklet is required.

8.3.1. All resuscitation trolleys/ grab bag must be maintained in a state of readiness at all times.

8.3.2 Weekly checks of the contents of the resuscitation trolley/grab bag should be carried out by two members of staff one of whom must be a registered nurse/practitioner and a unique numbered seal applied.

8.3.3 It is the responsibility of staff working in an area to familiarise themselves with the location of all resuscitation equipment.

8.3.4 Daily checks are required for defibrillators, oxygen cylinders and suction and the trolley seal has the correct number on it. Once complete the checks must be documented as having been done by filling in the correct box in the checking booklet.

8.4 Following the conclusion of a resuscitation attempt or if equipment has been used from the resuscitation trolley/grab bag, the trolley/grab bag should be checked, restocked and a unique numbered sealed tag applied.

8.4.1 Replacement items should be obtained via the area 'top-up' store or for specialised equipment from the emergency equipment cupboard located outside General Theatres. Site practitioners can be contacted out of hours.

8.4.2 The General Theatre receptionist holds the key to the cupboard in normal working hours; for out of hours contact, the theatre duty senior nurse (Bleep 801) or site practitioner.

8.5 A porter attends all arrest calls. They may be asked to obtain equipment not kept as standard on all resuscitation trolleys e.g. Capnography, EZIO, Autopulse, Patient Scoop.

8.6 Ward/department managers are responsible for ensuring that the trolley/grab bag checks are performed and for taking appropriate action if this has not been done.

8.6.1 They will also audit the checklists on a six monthly basis; this may be delegated to another member of the team.

8.7 All checklists will be audited by a member of the Resuscitation Team, at least annually.

8.8 Where a dedicated trolley/ grab bag is not required, basic emergency equipment should be available for staff to use until further help arrives.

8.9 The list of equipment to be stocked and checked must be agreed with the Resuscitation Officers.

8.10 All resuscitation equipment purchasing is subject to the organisation's standardisation strategy.

8.10.1 Each resuscitation trolley/ grab bag has standardised emergency drugs. Replacement drugs must be obtained immediately after use, or if the seal/box appears to have been tampered with, or if it is approaching expiry. In hours this can be done via pharmacy, otherwise contact site practitioners.

8.10.2 If drugs are missing, report the incident to Ward /Department manager/site practitioner and the Pharmacy department and complete a Datix incident reporting form.

8.11 Defibrillators are located strategically throughout the acute and community sites to enable shock delivery within 3 minutes of a patient's collapse.

8.11.1 The make and model of defibrillators should be standardised throughout the Trust. All emergency defibrillator equipment should be purchased with the consent and agreement of both the Resuscitation Group and Medical Engineering Manager.

8.11.2 Defibrillators should be checked in line with manufacturer's instructions by the relevant ward/departmental staff; any faults being reported to the Medical Engineering team.

8.12 Defibrillation will be performed using hands-free defibrillation pads.

8.13 While it is highly desirable that those who may be called upon to use an AED should be trained in their use, and keep their skills up to date, circumstances can dictate that no trained operator (or a trained operator whose certificate of training has expired) is present at the site of an emergency. Under these circumstances no inhibitions should be placed on any person willing to use an AED. (Resuscitaion Council (UK).

8.13.1 The Resuscitation Council (UK) advises that NHS Trusts should ensure that "norestriction is placed on the use of an AED by an untrained NHS employee confronted with a patient in cardiac arrest when no more highly trained individual is present. The administration of a defibrillatory shock should not be delayed waiting for more highly trained personnel to arrive. The same principle should apply to individuals whose period of qualification has expired".

8.14 **Resuscitation equipment not available on all trolleys/grab bags on Wonford site**

8.14.1 If intra-osseous (IO) access is required in the RD & E Wonford site there are IO drills and needles available from :-

- AMU
- CCU
- Clyst
- Emergency department
- Bramble
- Theatres
- Labour ward

8.14.2 Capnography units are located on:-

- AMU (West)
- Culm HDU
- ICU

8.14.3 CO2 monitoring is available on all ED defibrillators and the transfer defibrillator that is kept in the site practitioners' office.

8.14.4 Autopulse devices are located in:-

- CCU
- Emergency department (Resus)

8.14.5 Emergency tracheostomy equipment should be available in the following locations:-

- Beside the bed of all patients with a tracheostomy
- On ICU
- In ED Resus
- On Otter ward
- In the difficult airway trolley in theatre suites

## 9. CARDIAC ARREST WONFORD SITE

9.1 All telephones within the main building on the Wonford site may be used to dial 2222 in the event of a suspected cardiac arrest.

9.2 The cardiac arrest team is based within the main building of the Wonford site only.

9.3 Staff are advised to speak clearly and inform the operator they require the cardiac arrest team (stating adult cardiac arrest, paediatric cardiac arrest or maternal cardiac arrest) and give their location.

9.3.1 The operator will then immediately activate the appropriate team bleeps.

9.4 In the event of cardiac arrest in outlying buildings on the Wonford site off site, (9)999 should be dialled to summon an ambulance.

9.5 The Adult Cardiac arrest team comprises of the following personnel:-

- Medical ST 3 or above (or equivalent),
- F2/ST1 & F1 Doctor (on call for acute medicine)
- Anaesthetic/intensive care doctor or Advanced Critical Care Practitioner (ACCP)
- Site Practitioner (during normal; working hours if available, out of normal working hours and non-clinical areas only)
- Porter
- Resuscitation Officers (during normal working hours and if available)

9.6 If there is a return of spontaneous circulation/breathing (ROSC), the team should stay in attendance until the patient is transferred to the appropriate clinical area.

9.7 A 2222 Event Record audit form should be completed by the team leader or nominated deputy for every call at the time of the event.

## 10. CARDIAC ARREST AND MEDICAL EMERGENCY TEAM LEADER

10.1 The Medical ST3 or equivalent would normally assume the role as team leader.

10.2 The cardiac arrest team leader will have a current Resuscitation Council UK accredited Advanced Life Support qualification or paediatric equivalent, when appropriate.

10.3 The cardiac arrest team leader will co-ordinate the resuscitation process and will be responsible for decision making whilst including the team.

10.4 In the event of simultaneous or second cardiac arrests, the cardiac arrest team leader will designate who will respond to the emergency and obtain anaesthetic help by contacting the 2nd on call anaesthetist via switchboard.

## 11. ANAESTHETIC COVER FOR CARDIAC ARRESTS

11.1 The Anaesthetic department does not guarantee that an anaesthetist will be available for every cardiac arrest call. However; a representative from critical care will always attend (usually ICU doctor/ACCP).

## 12. PAEDIATRIC ARREST

12.1 In the event of a paediatric respiratory/cardiac arrest, staff should dial 2222 and state that it is a paediatric cardiac arrest/emergency and location where the team is required.

12.2 Switchboard will put out the call for the on-call paediatric emergency team.

12.3 Paediatric arrest/emergency team consists of:

- Paediatric Specialist Trainee (ST) (carry bleep 478)
- Paediatric SHO (carry bleep 479)
- Bramble SHO (Bleep 585)
- Most senior critical care doctor available (may be ICU doctor or anaesthetist)
- Paediatric Nurse Coordinator (bleep 603)
- Neonate Registrar (bleep 503)
- Site Practitioner
- Resuscitation officers (during normal working hours and if available)
- Porter

12.4 At least one member of the paediatric resuscitation team should possess a valid certificate in Advanced Paediatric Life Support (APLS) or European Paediatric Advanced Life Support (EPALS).

12.5 The role of the paediatric cardiac arrest team is to maintain cardiopulmonary resuscitation and to initiate advanced life support treatment (as per current Resuscitation Council guidelines) for as long as deemed clinically appropriate or necessary

12.6 A 2222 MET or cardiac arrest call should be recorded within the patients MyCare record as soon as possible after the resuscitation attempt.

12.7 Parents or carers of high-risk children will be offered resuscitation training.

## 13. NEONATAL EMERGENCY

13.1 In the event of a neonatal emergency, dial 2222 and clearly state that you require the neonatal emergency team and location where the team is required.

13.2 The switchboard will then immediately activate the neonatal emergency team bleeps.

13.3 **Neonatal emergency team consists of:**

- Neonatal ST4 or above Bleep 853
- Neonatal F2/ST1 Bleep 745
- Neonatal Nurse bleep holder 531
- Delivery suite co-ordinator bleep holder 529

13.4 The role of the neonatal emergency team is to maintain cardiopulmonary resuscitation and to initiate advanced life support treatment (as per current Resuscitation Council (UK) guidelines for as long as deemed clinically appropriate or necessary.

13.5 Ensure a record completion of the 2222 event is made in the patients MyCare record as soon as possible after the resuscitation attempt.

13.6 Training in newborn life support, assessment and recognition of the sick baby will be available for clinical staff that are expected to attempt resuscitation of neonates.

13.7 Parents on the Neonatal Unit are offered resuscitation training.

## 14. MATERNAL CARDIAC ARREST TEAM

14.1 If the individual is pregnant or recently delivered the caller should state 'Maternal cardiac arrest' within the RD & E Wonford site. The maternal cardiac arrest team is defined as:

- The adult cardiac arrest team as detailed above and also
- The on-call Obstetric registrar (764) and SHO (581)
- Labour ward co-ordinator (529)
- Obstetric consultant (525)
- Neonatal Registrar (853)
- Senior bleep holder neonatal nurse (529)
- Labour ward anaesthetist (981)
- Senior anaesthetist on call (484)

## 15. CARDIAC ARREST COMMUNITY SITES

15.1 If a patient has a cardiac arrest at a community hospital, clinic, health-centre/GP practice, patients home or any other site where staff may be working; staff must dial

(9)999 for an emergency ambulance, inform ambulance control of the location and commence CPR.

15.2 Defibrillation using an automated external defibrillator (AED) can be delivered by any member of staff alongside Basic Life Support by turning on the device and following the voice prompts.

15.2.1 A manual defibrillator can be used in AED mode by any member of staff by turning on the device and following the voice prompts. This is in line with Resuscitation Council (UK) recommendations.

15.2.2 Staff may only perform manual defibrillation if they have received the appropriate training and are competent to do so in their area of practice.

15.3 If the skill is not used in practice regularly, more frequent training should be sought.

15.4 Where resources allow, a member of the patient's own team should accompany the patient to the acute hospital. If staff cannot accompany the patient then phone 402317 and ask for the duty senior emergency doctor to discuss ongoing care.

15.5 There will be some people for whom attempting CPR is clearly inappropriate; for example, a person in the advanced stages of a terminal illness where death is imminent and unavoidable and CPR would not be successful, but for whom no formal CPR decision has been made and recorded. Also, there will be cases where healthcare professionals discover patients with features of irreversible death – for example, rigor mortis. In such circumstances, any healthcare professional that makes a carefully considered decision not to start CPR should be supported by their senior colleagues, employers and professional bodies. Hyper link to the document decsisions relationg to cardiopulmonary resuscitation section 8 p17 published 2016 Joint BMA, Resuscitation Council and Royal College of Nursing 3rd edition.

## 16. MEDICAL EMERGENCIES AND CARDIAC ARREST IN NON-CLINICAL AREAS

16.1 When a medical emergency or cardiac arrest occurs in a non-clinical area, hospital staff should provide instant resuscitation within the scope of their training/competence including basic life support, advanced life support and defibrillation. The Trust has a separate policy on this: Emergencies in non-clinical areas

16.2 Automated External Defibrillators (AEDs) have been installed in various locations around the Trust.

16.3 The location of these devices can be identified by an AED wall sign and easily recognizable white cabinets.

16.4 The cabinets also contain a bag valve mask (BVM).

16.5 When the cabinet door is opened, the alarm will sound for 30 seconds, and the lights will flash. The alarm will sound even if the door is closed and continues until turned off by a site practitioner or member of the general services staff. Site practitioners can be contacted via switchboard

16.6 All staff can use the defibrillators that are distributed in both the acute and community setting in an AED mode by turning on the device and following the voice prompts. As per Resuscitation Council (UK) guidance.

16.7 It is the staff member performing the defibrillation who is responsible for the safety of those around at all time.

16.8 The staff member must ensure that Resuscitation Council (UK) guidelines are adhered to when performing defibrillation.

## 17. SPECIAL CIRCUMSTANCES

### 17.1 Resuscitation of a patient with laryngectomy or tracheostomy

17.1.1 All staff involved in the care of patients with a laryngectomy or tracheostomy should be aware of the procedure for managing the patient's airway and breathing during resuscitation.

17.1.2 The National Tracheostomy Project provides effective guidelines for care of such patient groups.

17.1.3 Additional airway adjuncts must be kept on the resuscitation trolley in clinical areas caring for this patient group.

17.1.4 Patients admitted with a tracheostomy/ laryngectomy should have a box at their bedside with appropriate kit to be used in an emergency.

### 17.2 Anaphylaxis

17.2.1 All employees who are required to provide injections, immunisation programmes and other pharmacological interventions must be proficient in the recognition and immediate treatment protocols of anaphylaxis management.

17.2.2 Under Regulation 238 of the Human Medicines Regulation 2012 schedule 19 Up to 1 mg of Adrenaline 1:1000 can be given intramuscularly in two 500 mcg doses (each administered five minutes apart) for life saving intervention when anaphylaxis is is evident.

17.2.3 Registered practitioners are able to administer the adrenaline in accordance with their competency.

17.2.4 All other clinical staff must be proficient in the recognition of anaphylaxis and competent in raising the necessary alarm, i.e. the summoning of the Resuscitation Team, within the hospital setting and emergency ambulance in all other settings.

## 18. TREATMENT ESCALATION PLAN & RESUSCITATION DECISIONS

18.1 All patients, staff and visitors who have an unexpected cardiopulmonary arrest should be immediately resuscitated. The cardiac arrest team should be called unless an explicit and corroborated decision has already been made not to do so.

18.2 All decisions relating to CPR should be discussed with the patient or a patient's representative by a senior clinician and clearly documented.

18.3 Sometimes people who lack capacity to consent will require emergency medical treatment to save their life or prevent them from serious harm. In these situations, what steps are 'reasonable' will differ to those in non-urgent cases. In emergencies, it will almost always be in the person's best interests to give urgent treatment without delay. One exception to this is when the healthcare staff giving treatment are satisfied that an Advance Decision to Refuse Treatment exists, for example a Do not Attempt to Resuscitate (DNAR) Instruction or a Treatment Escalation Plan (TEP) has been agreed. See Advanced Decisions to Refuse Treatment Policy

18.4 If a person is being assessed for serious medical treatment and they lack capacity and are unbefriended, an independent mental capacity advocate (IMCA) must be instructed without delay. The only exception for the need to instruct an IMCA is if there is an emergency situation requiring an urgent decision to be made, for example, to save a person's life or for a resuscitation decision. However, once the emergency has passed, there is a duty to instruct an IMCA for any subsequent serious medical treatment decisions. Decisions regarding resuscitation are considered serious medical treatment and require an IMCA if there is no other advocate and the person lacks capacity. See Resuscitation Council (UK) guidance.

18.5. It is not Trust policy to use any single factor such as age or disability when making decisions regarding DNACPR. Clinicians should use all available information when making a DNACPR decision and should follow GMC guidance

18.6 If a clinician considers that CPR won't work the patient cannot require them to provide CPR. This said, the patient is still entitled to know that this decision has been made and the basis for it

Some individuals may have cultural or religious concerns regarding these descisions and discussions (e.g. concerns about 'choosing to die'). Staff have 24 hour access to chaplaincy services to support patients or relatives with ethical, religious or cultural concerns.

18.7 Once a decision relating to resuscitation and ceiling of treatment has been made a senior doctor should record that decision by completing a TEP ORDER within the MyCare record this will be recorded with an electronic signature

18.8 With the patients permission a 'DNACPR' decision should be explained to and discussed with the relatives as soon as is practical and this conversation documented on the digital TEP form in the patients record.

18.9 Any member of the medical or nursing teams looking after the patient who disagrees with a 'DNACPR' decision or escalation plan is encouraged to immediately communicate their concern to the most senior doctor or nurse available.

18.10 This person will take whatever immediate action is appropriate and refer the matter to the patient's consultant, who has the ultimate responsibility. If this is not appropriate or successful a second opinion should be sought and documented in the patients notes.

18.11 The decisions recorded on a TEP form should be reviewed if the patients clinical condition changes.

18.12 If appropriate, a new TEP form should be written and the previous one cancelled and filed at the back of the patient's notes.

18.13 A DNACPR order may be suspended for a procedure if appropriate - e.g. theatre. This should be communicated to the patient and next of kin and a review date/time set.

18.14 All staff involved with the patient should be aware of any changes to the DNACPR order.

18.15 On discharge any patient who has an active TEP ORDER (i.e. not the default) should receive a paper copy of that order (to be printed on discharge). In addition, a copy of the electronic TEP should be sent to the GP with the discharge summary

18.16 The patient (unless the senior clinician feels that this would harmful to the patient) and family/carers should be aware of the content of the TEP form.

18.17 All staff including ambulance crews should be aware of the patient's resuscitation status.

18.18 Patients who come into hospital with a TEP from the community:
The pre existing TEP decision should be recorded on the patients MyCare record by raising a TEP ORDER and recording on that the content of the pre-existing TEP. The original can also be scanned if wished and stored as a scanned document in the advanced care plan section on the patient record. Please note that a scaned TEP is NOT a substitute for raising a TEP ORDER which forms the basis for in hospital treatment escalation decisions.

The original TEP can be returned to the patient, unless the TEP decision changes during the hospital stay, under which circumstances, the patient should be given a copy of the new TEP on discharge.

18.19 If in doubt, always start resuscitation.

18.20 Mental Capacity Act Practice Guidance provides a legal framework for acting and making decisions on behalf of people aged 18 (in certain circumstances age 16) and those who lack the mental capacity to act or make decisions themselves. It provides clear guidelines for carers and professionals about who can take decisions, and in which situations. See Mental Capacity Act Practice Guidance.

## 19. RELATIVES WITNESSING RESUSCITATION ATTEMPTS

19.1 Relatives or next of kin should be offered the opportunity to stay during resuscitation attempts. Although this may be uncomfortable for medical and nursing staff, research suggests that it may help the grieving process if the patient subsequently dies. If relatives are present, a member of staff should be available throughout the process to support and explain the resuscitation to them. The relatives should be free to leave at any time.

## 20. POST RESUSCITATION CARE AND PATIENT TRANSFERS

20.1 The medical registrar remains responsible for the patient until the patient is transferred to a higher level of care or handed back to the original team. The medical registrar is also responsible for ensuring that the events and treatment are documented accurately in the patients' notes.

20.2 The Resuscitation Team Leader together with the patient's own doctors will decide the most appropriate care area for further management.

20.3 Safe transfer of the patient post cardiac arrest must be undertaken. From Community locations this will involve the ambulance service via (9)999. Within Wonford Main Hospital this should involve staff who has had training in the transfer of sick patients, usually a member of the critical care department.

20.4 All medical and nursing documentation must accompany the patient on transfer and a comprehensive verbal handover given to receiving team.

20.5 Portable equipment for safe transfer of the patient is available from the Intensive Care Units or Coronary Care Unit. There is a transfer monitor/defifbrillator held at the site practitioners office. This should be collected and checked prior to transfer of the patient, and should include capnography for intubated patients.

20.6 Oxygen for transfer can be obtained via the portering staff.

20.7 Best practice is for the staff involved in a resuscitation attempt to debrief afterwards. If a member of staff needs further support they can contact the resuscitation officers or Occupational Health department. A leaflet is also available 'Dealing with a critical Incident'

## 21. SAFETY

### 21.1 Manual Handling

21.1.1 In situations where the collapsed patient is on the floor, in a chair or in a restricted / confined space the organisational guidelines for the movement of the patient must be followed to minimise the risks of manual handling and related injuries to both staff and the patient. Also refer to the Resuscitation Council (UK) (2009) statement and the Trusts Moving and Handling Policy and Procedure (including bariatrics).

### 21.2 Infection Control

#### 21.2.1 Mouth-to-mouth ventilation

21.2.2 Isolated cases of infection to rescuer have been reported; therefore, mouth-to-mouth ventilation should not be performed within clinical settings.

21.2.3 A bag-valve-mask or pocket mask must be used.

21.2.4 Mouth-to mouth ventilation is contra-indicated, even outside clinical settings, in the following circumstances, when a pocket mask or bag-valve-mask MUST be used:

- All patients with evident facial trauma
- Evidence of non-intact oral mucosa - victim and/or rescuer
- Where oral mucosal integrity cannot be established
- All patients who are known to have an infectious disease.

21.2.5 Gloves should be worn by all participating in a resuscitation attempt.

21.2.6 Universal Precautions should be employed when dealing with body fluids (please see the Trust Infection Control and Decontamination Policy). Management of a resuscitation emergency is no exception.

21.2.7 When working in areas with limited access staff should ensure that emergency teams are able to reach them by unlocking doors.

21.2.8 Staff working in patients own home must ensure personal safety and follow Basic Life Support algorithm where possible. The staff member should dial 999 for assistance before commencing resuscitation. If the patient has a Treatment escalation plan or an advanced directive concerning limitations of treatment then the GP should be contacted.

## 22. ARCHIVING ARRANGEMENTS

The original of this policy will remain with the author. An electronic copy will be maintained on the Trust Intranet, (HUB) Archived electronic copies will be stored on the Trust's "archived policies" shared drive and will be held indefinitely.

## 23. PROCESS FOR MONITORING COMPLIANCE WITH AND EFFECTIVENESS OF THE POLICY

23.1 In order to monitor compliance with this policy, the following elements will be monitored:

| What areas need to be monitored? | How will this be evidenced? | Where will this be reported and by whom? |
|---|---|---|
| Cardiac arrest audit data will be collected with an audit form being completed after each resuscitation | Database maintained by the RO (Resuscitation Officer). | Via NCAA. The report of which will be discussed at the Resuscitation group |
| A qualitative review of 2222 calls will be performed on an on-going basis to identify quality and training issues. | Database maintained by the RO. | All results will be presented and discussed at the Resuscitation Group |
| Any specific learning outcomes and issues will be shared with the Governance Manager(s) for the appropriate division(s). | Datix incident forms completed by RO. Clinical Lead Resuscitation Group notified | These incidents will be investigated by divisions and discussed at divisional governance |
| Resuscitation equipment including trolleys and grab bags will be audited by the Resuscitation Department annually and results reviewed by the | Minutes of the Resuscitation Group meetings. | Any issues noted will be reported to the Patient Safety and Mortality Group |

| | | |
|---|---|---|
| Resuscitation Group, in order to identify areas of non-compliance and assist wards/departments to improve practice. | | |
| Directorates will monitor attendance rates for BLS training (adult, paediatric, neonatal) and put in place action plans to rectify any problems and notify the resuscitation officers. | ESR Records. | This will be monitored by the Resuscitation Team |
| The Resuscitation Officers will monitor the training carried out by key trainers. | Review of evaluation forms completed by attendees and attendance at an annual update. | The Resuscitation Group |
| Information on the analysis of audit data, benchmarking and the identification of areas of risk will form part of the resuscitation group reports to the Patient Safety Group | | Minutes of the Resuscitation Group and Clinical Effectiveness Committee meetings. |

## 24. REFERENCES


Department of Health (2000). *Health Service Circular HSC 2000/028: Resuscitation policy*. Available at: http://webarchive.nationalarchives.gov.uk/+/www.dh.gov.uk/en/Publicationsandstatistics/Lettersandcirculars/Healthservicecirculars/DH_4004244

Department of Health (2009*). Competencies for recognising and responding to acutely ill patients in hospital.* [online]. Available at: https://webarchive.nationalarchives.gov.uk/+/http://www.dh.gov.uk/en/Consultations/Closedconsultations/DH_083630

Care Quality Commission (2010). *Guidance about compliance - Essential standards of quality and safety*. [online]. Available at: http://www.cqc.org.uk/sites/default/files/documents/gac_-_dec_2011_update.pdf

Resuscitation Council [UK] (2010). *Resuscitation Guidelines 2010*. [online]. Available at: http://www.resus.org.uk/pages/guide.htm

Resuscitation Council [UK] (2013a). *Quality standards for cardiopulmonary resuscitation practice and training – Acute Care.* [online]. Available at: http://www.resus.org.uk/pages/QSCPR_Acute.pdf

Resuscitation Council [UK]. (2013b). *Equipment and drug lists for cardiopulmonary resuscitation - primary care.* [online]. Available at: https://www.resus.org.uk/quality-standards/primary-care-quality-standards-for-cpr/

Resuscitation Council [UK]. Available at: http://www.resus.org.uk/

European Resuscitation Council. Available at:
https://www.erc.edu/

National Institute for Health and Care Excellence (2007). NICE guideline CG50: *Acutely ill patients in hospital: Recognition of and response to acute illness in adults in hospital.* [online]. Available at: http://www.nice.org.uk/guidance/cg50

*The Mental Capacity Act 2005. (c.9)* London: Stationery Office. Available at:
http://www.legislation.gov.uk/ukpga/2005/9/contents

Office of the Public Guardian (2007). *Guidance: Independent Mental Capacity Advocates.* [online]. Available at:
https://www.gov.uk/government/publications/independent-mental-capacity-advocates

*The Human Medicines Regulations 2012 (SI 1916).* London: Stationery Office. Available at: http://www.legislation.gov.uk/uksi/2012/1916/regulation/238/made

Medicines and Healthcare Products Regulatory Agency (2014). *Administration of injectable medicines for the purpose of saving life in an emergency*. [online]. Available at:
http://webarchive.nationalarchives.gov.uk/20141205150130/http://www.mhra.gov.uk/Howweregulate/Medicines/Availabilityprescribingsellingandsupplyingofmedicines/ExemptionsfromMedicinesActrestrictions/Administrationofinjectablemedicinesforthepurposeofsavinglifeinanemergency/index.htm

Intensive Care and National Audit and Research Centre (n.d). Available at:
https://www.icnarc.org/

One Chance To Get It Right. Available at:
https://www.gov.uk/government/uploads/system/uploads/attachment_data/file/323188/One_chance_to_get_it_right.pdf

## 25. FURTHER INFORMATION

Information on Hub regarding the Mental Capacity Act
Information on Hub regarding End of Life Care
TEP form Sample

## APPENDIX 1: COMMUNICATION PLAN

Royal Devon and Exeter NHS
NHS Foundation Trust

### COMMUNICATION PLAN

The following action plan will be enacted once the document has gone live.

| | |
|---|---|
| **Staff groups that need to have knowledge of the policy** | All staff |
| **The key changes if a revised policy** | Addition of recognition and escalation of the deteriorating patient usings NEWS2<br>Addition of Cardiac arrest management in community setting.<br>No inhibitions placed on staff regarding use of AED defibrillator<br>Nil – entirely restructured<br>To advise all staff of their responsibilities and actions involved with resuscitation and the management of the deteriorating patient across the Trust |
| **How new staff will be made aware of the policy and manager action** | Via the corporate induction process.<br>Communication via the Hub |
| **Specific Issues to be raised with staff** | Staff will be made aware on induction and through mandatory training of the new resuscitation policy highlighting key points as above in Key points section. |
| **Training available to staff** | Resuscitation Department, mandatory updates, Resuscitation Council (UK) courses, local bespoke education by key trainers |
| **Any other requirements** | Nil |
| **Issues following Equality Impact Assessment (if any)** | Nil |
| **Location of hard / electronic copy of the document etc.** | The original of this policy will remain with the author. An electronic copy will be maintained on the Trust Intranet, (HUB) Archived electronic copies will be stored on the Trust's “archived policies” shared drive and will be held indefinitely. |

## APPENDIX 2: EQUALITY IMPACT ASSESSMENT TOOL

| | |
|---|---|
| **Name of document** | Cardiopulmonary Resuscitation Policy |
| **Division/Directorate and service area** | Safety and Risk |
| **Name, job title and contact details of person completing the assessment** | Donna Seccombe, Senior Resuscitation Officer |
| **Date completed:** | March 2019 |

**The purpose of this tool is to:**

- **identify** the equality issues related to a policy, procedure or strategy
- **summarise the work done** during the development of the document to reduce negative impacts or to maximise benefit
- **highlight unresolved issues** with the policy/procedure/strategy which cannot be removed but which will be monitored, and set out how this will be done.

**1. What is the main purpose of this document?**

This policy provides staff with guidance on the responsibilities of the organisation, resuscitation service and staff in the recognition, escalation and mangement of deteriorating patients and in the management of a cardiopulmonary arrest. This guidance is as recommended by the Resuscitation Council (UK). This policy applys to all staff within the organisation in the acute and community setting.

**2. Who does it mainly affect?** *(Please insert an "x" as appropriate:)*

Carers ☒ Staff ☒ Patients ☒ Other (please specify)visitors

**3. Who might the policy have a 'differential' effect on, considering the "protected characteristics" below?** (By *differential* we mean, for example that a policy may have a noticeably more positive or negative impact on a particular group e.g. it may be more beneficial for women than for men)

***Please insert an "x" in the appropriate box*** *(x)*

| Protected characteristic | Relevant | Not relevant |
|---|---|---|
| Age | ☒ | ☐ |
| Disability | ☒ | ☐ |
| Sex - *including: Transgender, and Pregnancy / Maternity* | ☐ | ☒ |
| Race | ☐ | ☒ |
| Religion / belief | ☒ | ☐ |

| | | |
|---|---|---|
| Sexual orientation – *including: Marriage / Civil Partnership* | ☐ | ☒ |

4. **Apart from those with protected characteristics, which other groups in society might this document be particularly relevant to…** (e.g. those affected by homelessness, bariatric patients, end of life patients, those with carers etc.)?

| |
|---|
| Homeless may not have up to date medical notes. |

5. **Do you think the document meets our human rights obligations?** ☐

*Feel free to expand on any human rights considerations in question 6 below.*

| ***A quick guide to human rights:*** |
|---|
| • ***Fairness –*** *how have you made sure it treat everyone justly?*<br>• ***Respect –*** *how have you made sure it respects everyone as a person?*<br>• ***Equality –*** *how does it give everyone an equal chance to get whatever it is offering?*<br>• ***Dignity –*** *have you made sure it treats everyone with dignity?*<br>• ***Autonomy –*** *Does it enable people to make decisions for themselves?* |

**6. Looking back at questions 3, 4 and 5, can you summarise what has been done during the production of this document and your consultation process to support our equality / human rights / inclusion commitments?**

| |
|---|
| It was noted during the review of the policy that age, disability, religious belief or cultural beliefs may be relevant to the policy. We have ammeded the policy to make such issues explicit, to ensure these are proactively addressed. |

**7. If you have noted any 'missed opportunities', or perhaps noted that there remains some concern about a potentially negative impact** please note this below and how this will be monitored/addressed.

| | |
|---|---|
| **"Protected characteristic":** | |
| **Issue:** | |
| **How is this going to be monitored/ addressed in the future:** | |
| **Group that will be responsible for ensuring this carried out:** | |